Patient Name	: Mr.PUSHKAR CHAUHAN	Collected	: 29/Sep/2026 09:13AM
Age/Gender	: 34 Y 7 M 26 D/M	Received	: 29/Sep/2026 04:09PM
UHID/MR No	: APJ1.0021721223	Reported	: 29/Sep/2026 04:29PM
Visit ID	: CPPHOPV196	Status	: Final Report
Ref Doctor	: Self	Centre Name	: PRESTIGE PRIMROSE HILLS - SOCI

DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
COMPLETE BLOOD COUNT (CBC) , *WHOLE BLOOD EDTA*				
HAEMOGLOBIN	14.6	g/dL	13-17	Spectrophotometer
PCV	42.70	%	40-50	Electronic pulse & Calculation
RBC COUNT	5.38	Million/cu.mm	4.5-5.5	Electrical Impedence
MCV	**79.3**	fL	83-101	Calculated
MCH	27.1	pg	27-32	Calculated
MCHC	34.1	g/dL	31.5-34.5	Calculated
RDW-CV	13.6	%	11.6-14	Calculated
TOTAL LEUCOCYTE COUNT (TLC)	4,330	cells/cu.mm	4000-10000	Electrical Impedance
DIFFERENTIAL LEUCOCYTIC COUNT (DLC)				
NEUTROPHILS	45	%	40-80	Flow cytometry
LYMPHOCYTES	**43**	%	20-40	Flow cytometry
EOSINOPHILS	3	%	1-6	Flow cytometry
MONOCYTES	9	%	2-10	Flow cytometry
BASOPHILS	0	%	0-2	Flow cytometry
CORRECTED TLC	4,330	Cells/cu.mm		Calculated
ABSOLUTE LEUCOCYTE COUNT				
NEUTROPHILS	**1948.5**	Cells/cu.mm	2000-7000	Calculated
LYMPHOCYTES	1861.9	Cells/cu.mm	1000-3000	Calculated
EOSINOPHILS	129.9	Cells/cu.mm	20-500	Calculated
MONOCYTES	389.70	Cells/cu.mm	200-1000	Calculated
Neutrophil lymphocyte ratio (NLR)	1.05		0.78- 3.53	Calculated
PLATELET COUNT	221000	cells/cu.mm	150000-410000	Electrical impedence
MPV	10	Fl	8.1-13.9	Calculated

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SIN No:PPH260900160

This test has been performed at Apollo Health and Lifestyle ltd- RRL Bangalore , Madiwala.

Patient Name	: Mr.PUSHKAR CHAUHAN	Collected	: 29/Sep/2026 09:13AM
Age/Gender	: 34 Y 7 M 26 D/M	Received	: 29/Sep/2026 04:09PM
UHID/MR No	: APJ1.0021721223	Reported	: 29/Sep/2026 05:04PM
Visit ID	: CPPHOPV196	Status	: Final Report
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DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
ESR , *WHOLE BLOOD EDTA*	**17**	mm at the end of 1 hour	0-15	Kinetic photometry

Comment:

1. ESR is an acute phase reactant which indicates presence and severity of an inflammatory process. ESR is not diagnostic of any specific disease. ESR is a screening test with low sensitivity. CRP has better sensitivity than ESR.
2. ESR Test is used to monitor the course or response to treatment of certain diseases.
3. ESR is an indirect measure of the extent of inflammation (infections, autoimmune disorders, certain anemias & malignancies).
4. There are many dieases such as typhoid in which ESR is not increased.

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Patient Name	: Mr.PUSHKAR CHAUHAN	Collected	: 29/Sep/2026 09:13AM
Age/Gender	: 34 Y 7 M 26 D/M	Received	: 29/Sep/2026 03:49PM
UHID/MR No	: APJ1.0021721223	Reported	: 29/Sep/2026 04:23PM
Visit ID	: CPPHOPV196	Status	: Final Report
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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
GLUCOSE, FASTING , *NAF PLASMA*	91	mg/dL	70-100	Hexokinase

Comment:

As per American Diabetes Guidelines, 2023

Fasting Glucose Values in mg/dL	Interpretation
70-100 mg/dL	Normal
100-125 mg/dL	Prediabetes
≥126 mg/dL	Diabetes
<70 mg/dL	Hypoglycemia

Note:

1.The diagnosis of Diabetes requires a fasting plasma glucose of > or = 126 mg/dL and/or a random / 2 hr post glucose value of > or = 200 mg/dL on at least 2 occasions.

2. Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis & is considered critical.

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SIN No:PPH260900159

Patient Name	: Mr.PUSHKAR CHAUHAN	Collected	: 29/Sep/2026 09:13AM
Age/Gender	: 34 Y 7 M 26 D/M	Received	: 29/Sep/2026 03:35PM
UHID/MR No	: APJ1.0021721223	Reported	: 29/Sep/2026 04:19PM
Visit ID	: CPPHOPV196	Status	: Final Report
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Test Name	Result	Unit	Bio. Ref. Interval	Method
HBA1C (GLYCATED HEMOGLOBIN) , *WHOLE BLOOD EDTA*				
HBA1C, GLYCATED HEMOGLOBIN	5.6	%		HPLC
ESTIMATED AVERAGE GLUCOSE (eAG)	114	mg/dL		Calculated

Comment:

Reference Range as per American Diabetes Association (ADA) 2023 Guidelines:

Reference Group	HbA1c (%)	HbA1c (mmol/mol)
Non-Diabetic	<5.7	<38.8
Prediabetes	5.7 – 6.4	38.8 – 46.5
Diabetes	≥ 6.5	≥47.5
Diabetics		
Excellent control	6 – 7	42.1 – 53.0
Fair to good control	7 – 8	53.0 – 63.9
Unsatisfactory control	8 – 10	63.9 – 85.8
Poor control	>10	>85.8
Note: HbA1c IFCC (mmol/mol) = (10.93 x HbA1c NGSP (%)) – 23.50		

1. HbA1C is recommended by American Diabetes Association for Diagnosing Diabetes and monitoring Glycemic Control by American Diabetes Association guidelines 2023.
2. Trends in HbA1c values is a better indicator of Glycemic control than a single test.
3. Low HbA1c in Non-Diabetic patients are associated with Anemia (Iron Deficiency/Hemolytic), Liver Disorders, Chronic Kidney Disease. Clinical Correlation is advised in interpretation of low Values.
4. Falsely low HbA1c (below 4%) may be observed in patients with clinical conditions that shorten erythrocyte life span or decrease mean erythrocyte age. HbA1c may not accurately reflect glycemic control when clinical conditions that affect erythrocyte survival are present.
5. In cases of Interference from Haemoglobin variants (HbF >25%, Homozygous Hemoglobinopathies) in HbA1C testing, alternative methods (Fructosamine) estimation is recommended for Glycemic Control. Abnormal Haemoglobin studies (HPLC/Electrophoresis) is recommended for detection of Hemoglobinopathies.

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SIN No:PPH260900162

Patient Name	: Mr.PUSHKAR CHAUHAN	Collected	: 29/Sep/2026 09:13AM
Age/Gender	: 34 Y 7 M 26 D/M	Received	: 29/Sep/2026 04:08PM
UHID/MR No	: APJ1.0021721223	Reported	: 29/Sep/2026 05:26PM
Visit ID	: CPPHOPV196	Status	: Final Report
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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
LIPID PROFILE , *SERUM*				
TOTAL CHOLESTEROL	**207**	mg/dL	< 200	CHOD-PAD
TRIGLYCERIDES	108	mg/dL	< 150	GPO-PAP
HDL CHOLESTEROL	42	mg/dL	>=40 Desirable	Enzymatic Immunoinhibition
NON-HDL CHOLESTEROL	**165**	mg/dL	<130	Calculated
LDL CHOLESTEROL	**143.6**	mg/dL	<100	Calculated (Friedewald)
VLDL CHOLESTEROL	21.6	mg/dL	<30	Calculated
CHOL / HDL RATIO	4.95		0-4.97	Calculated
ATHEROGENIC INDEX (AIP)	0.050		<0.11	Calculated

Comment:

Reference Interval as per National Cholesterol Education Program (NCEP) Adult Treatment Panel III Report.
Below Table as per Lipid Association of India (LAI) (2023) and Cardiological Society of India (CSI) (2024) Guidelines and Consensus Statements for Dyslipidemia Management:

	Low Risk	Moderate Risk	High Risk	Very High Risk	Extremely High Risk
Total Cholesterol	< 200	< 200	200 – 239	≥ 240	≥ 240
Triglycerides	<150	< 150	150 – 199	200 – 499	≥ 500
LDL	< 100	100 – 129	130 – 159 Target for High Risk: < 70	160 – 189 Target for Very High Risk: < 50	≥ 190 Target for Extreme Risk – Category A,B :< 30,Category C: 10 – 15
HDL	≥ 60	≥ 60	M: <40, F: <50	<30	<30
Non-HDL Cholesterol	< 130	130 – 159	160-189 Target for High Risk: < 100	190 – 219 Target for Very High Risk: < 80	≥220 Target for Extreme Risk – Category A,B :< 60, Category C: 40 – 45

Note: **Low risk** – No known risk factor of cardiovascular disease.**Moderate risk** – Any one risk factor eg;smoking/hypertension/diabetes mellitus etc.
High risk – Two or more risk factors without any disease manifestation, chronic kidney disease, long-standing diabetes mellitus existing for >10 years,history of heterozygous familial hypercholesterolemia. **Very high risk** – clinical evidence of coronary artery disease, long-standing diabetes mellitus existingfor >20 years, etc. **Extremely high risk** – recurrent vascular events.
1. Measurements for Lipids (Especially Triglycerides) can show physiological (Dependent on diet,10-12 hrs fasting pre-test condition) & analytical variations.
2. Lipid Association of India (LAI) recommends screening of all adults (>20 yrs) for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors with lipid profile testing. The association recommends testing also to include Apolipoprotein B & Lipoprotein (a) for stratification and defining LDL – C targets.

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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
LIVER FUNCTION TEST (LFT) , *SERUM*				
BILIRUBIN, TOTAL	0.76	mg/dL	0-1.2	Diazo
BILIRUBIN CONJUGATED (DIRECT)	0.19	mg/dL	0-0.3	Diazo
BILIRUBIN (INDIRECT)	0.57	mg/dL	0.0-1.1	Calculated
ALANINE AMINOTRANSFERASE (ALT/SGPT)	**73.6**	U/L	10-50	IFCC with Pyridoxal Phosphate
ASPARTATE AMINOTRANSFERASE (AST/SGOT)	47.4	U/L	10-50	IFCC with Pyridoxal Phosphate
AST (SGOT) / ALT (SGPT) RATIO (DE RITIS)	0.6		<1.15	Calculated
ALKALINE PHOSPHATASE	73.10	U/L	40-129	IFCC
PROTEIN, TOTAL	7.49	g/dL	6.4-8.3	Biuret
ALBUMIN	4.63	g/dL	3.5-5.2	Bromo Cresol Green
GLOBULIN	2.86	g/dL	2.0-3.5	Calculated
A/G RATIO	1.62		0.9-2.0	Calculated

Comment:

LFT results reflect different aspects of the health of the liver, i.e., hepatocyte integrity (AST & ALT), synthesis and secretion of bile (Bilirubin, ALP), cholestasis (ALP, GGT), protein synthesis (Albumin) Common patterns seen:

1. Hepatocellular Injury: *AST – Elevated levels can be seen. However, it is not specific to liver and can be raised in cardiac and skeletal injuries.*ALT – Elevated levels indicate hepatocellular damage. It is considered to be most specific lab test for hepatocellular injury. Values also correlate well with increasing BMI. Disproportionate increase in AST, ALT compared with ALP. AST: ALT (ratio) – In case of hepatocellular injury AST: ALT > 1In Alcoholic Liver Disease AST: ALT usually >2. This ratio is also seen to be increased in NAFLD, Wilsons's diseases, Cirrhosis, but the increase is usually not >2.Note- If both SGPT and SGOT are within reference range then AST:ALT (De Ritis ratio) does not have any clinical significance.
2. Cholestatic Pattern:*ALP – Disproportionate increase in ALP compared with AST, ALT. ALP elevation also seen in pregnancy, impacted by age and sex.*Bilirubin (Direct) and GGT elevated- helps to establish hepatic origin.
3. Synthetic function impairment:*Albumin- Liver disease reduces albumin levels, Correlation with PT (Prothrombin Time) helps.
4. Associated tests for assessment of liver fibrosis - Fibrosis-4 and APRI Index.

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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
RENAL PROFILE/KIDNEY FUNCTION TEST (RFT/KFT) , *SERUM*				
CREATININE	**1.37**	mg/dL	0.7-1.2	Jaffe
.eGFR - ESTIMATED GLOMERULAR FILTRATION RATE	69.14	mL/min/1.73m²	>60	CKD-EPI Formula 2021
UREA	32.40	mg/dL	13-43	Urease
BLOOD UREA NITROGEN	15.1	mg/dL	8.0 - 23.0	Calculated
URIC ACID	5.10	mg/dL	3.5-7.2	Uricase
CALCIUM	10.00	mg/dL	8.6-10	NM-Bapta
PHOSPHORUS, INORGANIC	2.62	mg/dL	2.5-4.5	Phosphomolybdate Complex
SODIUM	139.4	mmol/L	136-145	ISE (Indirect)
POTASSIUM	5.05	mmol/L	3.5-5.1	ISE (Indirect)
CHLORIDE	103.3	mmol/L	98-107	ISE (Indirect)
PROTEIN, TOTAL	7.49	g/dL	6.4-8.3	Biuret
ALBUMIN	4.63	g/dL	3.5-5.2	Bromo Cresol Green
GLOBULIN	2.86	g/dL	2.0-3.5	Calculated
A/G RATIO	1.62		0.9-2.0	Calculated

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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
CALCIUM , *SERUM*	10.00	mg/dL	8.6-10	NM-Bapta

Comments:-

Serum calcium measurements are done to monitor and diagnose disorders of skeletal system, parathyroid gland, kidney, muscular disorders, and abnormal vitamin D and protein levels.

Test Name	Result	Unit	Bio. Ref. Interval	Method
C-REACTIVE PROTEIN CRP (QUANTITATIVE) , *SERUM*	3.98	mg/L	0-5	Latex Particle Immunoturbidimetric

Comment:

C-reactive protein (CRP) is one of the most sensitive acute-phase reactants for inflammation. Measuring changes in the concentration of CRP provides useful diagnostic information about the level of acuity and severity of a disease. Unlike ESR, CRP levels are not influenced by hematologic conditions such as anemia, polycythemia etc.

Increased levels are consistent with an acute inflammatory process. After onset of an acute phase response, the serum CRP concentration rises rapidly (within 6-12 hours and peaks at 24-48 hours) and extensively.Concentrations above 100 mg/L are associated with severe stimuli such as major trauma and severe infection (sepsis).

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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
IRON STUDIES (IRON + TIBC) , *SERUM*				
IRON	191.0	µg/dL	33-193	FerroZine
TOTAL IRON BINDING CAPACITY (TIBC)	309	ug/dL	250-400	Dye Binding
UNSATURATED IRON BINDING CAPACITY (UIBC)	**118.00**	ug/dL	125-345	Direct Estimation by Ferrozine
% OF TRANSFERRIN SATURATION	**61.81**	%	14-50	Calculated

Comment:

Transferrin is the primary plasma iron transport protein, which binds iron strongly at physiological pH. Transferrin is generally only 25% to 30% saturated with iron. The additional amount of iron that can be bound is the unsaturated iron-binding capacity (UIBC). Diurnal variation is seen in serum iron levels—normal values in midmorning, low values in midafternoon, very low values (approximately 10 µg/dL) near midnight.

TIBC measures the blood's capacity to bind iron with transferrin (TRF). Estrogens and oral contraceptives increase TIBC levels. Asparaginase, chloramphenicol, corticotropin, cortisone, and testosterone decrease the TIBC levels.

% saturation represents the amount of iron-binding sites that are occupied. Iron saturation is a better index of iron stores than serum iron alone. % saturation is decreased in iron deficiency anemia (usually <10% in established deficiency).

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Patient Name	: Mr.PUSHKAR CHAUHAN	Collected	: 29/Sep/2026 09:13AM
Age/Gender	: 34 Y 7 M 26 D/M	Received	: 29/Sep/2026 03:45PM
UHID/MR No	: APJ1.0021721223	Reported	: 29/Sep/2026 04:40PM
Visit ID	: CPPHOPV196	Status	: Final Report
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DEPARTMENT OF IMMUNOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
THYROID PROFILE TOTAL (T3, T4, TSH) , *SERUM*				
TRI-IODOTHYRONINE (T3, TOTAL)	120	ng/dL	84.6-202	ECLIA
THYROXINE (T4, TOTAL)	7.23	µg/dL	5.12-14.06	ECLIA
TSH (Ultrasensitive/4thGen)	1.690	µIU/mL	0.270-4.20	ECLIA

Comment:

For pregnant females	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 – 3.0
Third trimester	0.3 – 3.0

1. TSH is a glycoprotein hormone secreted by the anterior pituitary. TSH activates production of T3 (Triiodothyronine) & its prohormone T4 (Thyroxine). Increased blood level of T3 and T4 inhibit production of TSH.
2. TSH is elevated in primary hypothyroidism and will be low in primary hyperthyroidism. Elevated or low TSH in the context of normal free thyroxine is often referred to as sub-clinical hypo- or hyperthyroidism respectively.
3. Both T4 & T3 provides limited clinical information as both are highly bound to proteins in circulation and reflects mostly inactive hormone. Only a very small fraction of circulating hormone is free and biologically active. Isolated low T3 is often noticed in elderly
4. Significant variations in TSH can occur with circadian rhythm, hormonal status, stress, sleep deprivation, medication & circulating antibodies.

TSH	T3	T4	FT4	Possible Suggested Conditions correlating with pattern
High	Low	Low	Low	Primary Hypothyroidism, Post Thyroidectomy, Chronic Autoimmune Thyroiditis
High	N	N	N	Subclinical Hypothyroidism, Autoimmune Thyroiditis, Insufficient Hormone Treatment.
N/Low	Low	Low	Low	Secondary and Tertiary Hypothyroidism
Low	High	High	High	Primary Hyperthyroidism, Goitre, Thyroiditis, Drug effects, Early Pregnancy
Low	N	N	N	Subclinical Hyperthyroidism
Low	Low	Low	Low	Central Hypothyroidism, Treatment with Hyperthyroidism
Low	N	High	High	Thyroiditis, Interfering Antibodies
N/Low	High	N	N	T3 Thyrotoxicosis, Non thyroidal causes
High	High	High	High	Pituitary Adenoma; TSHoma/Thyrotropinoma

DR.Lucky Sinha
M.B.B.S,M.D(Pathology)
Consultant pathologist

SIN No:PPH260900161

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Patient Name	: Mr.PUSHKAR CHAUHAN	Collected	: 29/Sep/2026 09:13AM
Age/Gender	: 34 Y 7 M 26 D/M	Received	: 29/Sep/2026 03:45PM
UHID/MR No	: APJ1.0021721223	Reported	: 29/Sep/2026 04:49PM
Visit ID	: CPPHOPV196	Status	: Final Report
Ref Doctor	: Self	Centre Name	: PRESTIGE PRIMROSE HILLS - SOCI

DEPARTMENT OF IMMUNOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
VITAMIN D (25 - OH VITAMIN D) , *SERUM*	**22.8**	ng/mL	30-100	ECLIA

Comment:

BIOLOGICAL REFERENCE RANGES

VITAMIN D STATUS	VITAMIN D 25 HYDROXY (ng/mL)
DEFICIENCY	<10
INSUFFICIENCY	10 – 30
SUFFICIENCY	30 – 100
TOXICITY	>100

The biological function of Vitamin D is to maintain normal levels of calcium and phosphorus absorption. 25-Hydroxy vitamin D is the storage form of vitamin D. Vitamin D assists in maintaining bone health by facilitating calcium absorption. Vitamin D deficiency can also cause osteomalacia, which frequently affects elderly patients.

Vitamin D Total levels are composed of two components namely 25-Hydroxy Vitamin D2 and 25-Hydroxy Vitamin D3 both of which are converted into active forms. Vitamin D2 level corresponds with the exogenous dietary intake of Vitamin D rich foods as well as supplements. Vitamin D3 level corresponds with endogenous production as well as exogenous diet and supplements. Vitamin D from sunshine on the skin or from dietary intake is converted predominantly by the liver into 25-hydroxy vitamin D, which has a long half-life and is stored in the adipose tissue. The metabolically active form of vitamin D, 1,25-di-hydroxy vitamin D, which has a short life, is then synthesized in the kidney as needed from circulating 25-hydroxy vitamin D. The reference interval of greater than 30 ng/mL is a target value established by the Endocrine Society.

Decreased Levels:- Inadequate exposure to sunlight, Dietary deficiency, Vitamin D malabsorption, Severe Hepatocellular disease., Drugs like Anticonvulsants, Nephrotic syndrome.

Increased levels:- Vitamin D intoxication.

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Test Name	Result	Unit	Bio. Ref. Interval	Method
VITAMIN B12 , *SERUM*	726	pg/mL	197-771	ECLIA

Comment:

Population based data reflecting exact scenario of vitamin B12 levels in Indian population is still evolving, however, different studies reporting a deficiency in adults, pregnant women and children ranging from 16% to 77% with average of about 47%. This high incidence is attributed to vegetarian food habits of large majority of Indian population.

Vitamin B12 deficiency frequently causes macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination, and affective behavioral changes. A significant increase in RBC MCV may be an important indicator of vitamin B12 deficiency. B12 levels in the range of 150 to 190 pg/ml may not be associated with any clinical manifestations, while B12 levels below 100 pg/ml are often associated with clinical symptoms. However, for an individual based on other co-morbid conditions or other nutritional deficiency (especially folate) the manifestations can vary accordingly.

If clinical symptoms suggest deficiency, measurement of active vitamin B12, MMA and homocysteine should be considered as further workup.

Test Name	Result	Unit	Bio. Ref. Interval	Method
FERRITIN , *SERUM*	55.6	ng/mL	30-400	ECLIA

Comment:

- Ferritin estimation is useful in the diagnosis of iron deficiency anemia and iron overload.
- Increased levels seen in hemachromatosis, frequent blood transfusions with packed RBCs and alcoholic liver disease.
- Decreased levels seen in heavy menstrual bleeding, poor absorption of iron, iron deficiency anaemia and long term GI bleed.
- Ferritin is an acute phase reactant and thus may be increased with inflammation, chronic infection, liver disease, auto-immune disorders and some type of cancers. Ferritin is not used to detect or monitor these conditions.

Test Name	Result	Unit	Bio. Ref. Interval	Method
TOTAL PROSTATIC SPECIFIC ANTIGEN (tPSA) , *SERUM*	0.580	ng/mL	< 1.4	ECLIA

Comment:

- Prostate specific antigen (PSA) is a glycoprotein having a close structural relationship to the glandular kallikrein & functions as a serine proteinase. PSA about 70-90% circulates in blood by

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Consultant pathologist

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formation of complexes with protease inhibitors (such as alpha 1 antichymotrypsin (ACT), alpha 2 macroglobulin), while about remaining 10 30 % of PSA in blood circulates in the free form, but is proteolytically inactive.

- Elevated levels of serum PSA can be associated with various benign prostatic conditions. Infection, trauma, inflammation, and benign prostatic hyperplasia can elevate serum PSA levels. Transient increase in PSA can also be seen following per rectal digital or sonological examinations. For results between 4-10 ng/ml, free PSA & free/total ratio is recommended.
- PSA measurements are also used in the monitoring of therapy in cancer patients.

DR.Lucky Sinha
M.B.B.S,M.D(Pathology)
Consultant pathologist

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Patient Name	: Mr.PUSHKAR CHAUHAN	Collected	: 29/Sep/2026 09:13AM
Age/Gender	: 34 Y 7 M 26 D/M	Received	: 29/Sep/2026 03:59PM
UHID/MR No	: APJ1.0021721223	Reported	: 29/Sep/2026 05:02PM
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DEPARTMENT OF SEROLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
RHEUMATOID FACTOR (RA) , *SERUM*	NEGATIVE		NEGATIVE	LATEX AGGLUTINATION

Comment:

This is a latex agglutination test for qualitative detection of rheumatoid factors of IgM class.
Kit sensitivity : 10 IU/ml

In rheumatoid arthritis, diagnostically useful autoantibodies termed as 'Rheumatoid Factors' (RF) can be detected.
Positive test indicates presence of rheumatoid factors in the test specimen. RF is not specific only for Rheumatoid arthritis, but it is often seen in cases of chronic infection and other systemic inflammatory conditions.

***** End Of Report *****

Dr.Dinesh Gopal
PhD(Medical Microbiology)
Consultant Microbiologist

SIN No:PPH260900158

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